جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : انشراح عزوز محمود الليثى
المحافظة : القاهره **الحي / المدينة :** الخليفه **العنوان :** ..
رقم البطاقة : 24710030101624 **رقم التليفون :** 01100305631 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 03-10-1947 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: انشراح عزوز محمود الليثى
National ID: 24710030101624

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



DMS

مقاعية



National ID Front & Back (same page)
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مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

04/05/2026

MDT FORM

Name:	Arshiah Eazuz Mohamoud Elisy		
Age:	79 Y	MRN:	6699
Menopausal Sta	POST	ID:	24710030101624
NRS SCORE:	PS_2	Phone:	01004235196
BMI:	35	Marital Status:	MARRIED
CO Morbidity:	HTN AND CARDIAC NOT ON FOLLOW UP	Family History:	+VE
		Reason Of MDT:	NEW CASE

Clinical Data:

BX: IDC G2

ER: 8/8 PR: 0/8 HER2: 0 KI67: 20

Sonomammography:
AT BREAST LOQ MASS AT 6 CLOCK 3*2.8 .... ACR - A
FEW LNS WITH INCREASED CORTICAL THICKNESS >3MM LARGEST 1.3*1.3
CLINICALY INCREASED SKIN THICKNESS AND RETRACTED NAC AND SKIN

Mets Workup:
CT CAP WITH C
BIL FEW PULMONARY NODULES FOR FOLLOW UP
PROMINANT PULMONARY TRUNCK AND MILD CARDIOMEGALY (ECHO REQ)
DILATED CBD WITH NO OBSTR OR JAUNDICE
SMALL HIATUS HERNIA

History:
CLINICALLY T4B N2 FOCAL SKIN THICKNESS OVER THE MASS

Recommendations:
CARDIAC CONSULTATION
AND FOR POSSIPLE UPFRONT SURGERY AS PS NOT WELL FOR NACT

MDT Outcome & Plan
AS EF 62% AND THE CASE IS LOCALLY ADBVANCED THERE IS A CLINICAL BENIFIT OF NACT SO FOR NACT EC X3 TAXOL WEEKLING AFTER COUNCLING OF THE PT AND SEE THE PERFORMANCE STATUS

R L

TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: انشراح عزوز محمود الليثي	
MR. : 6699	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a history of double primary malignancies. Thirteen years ago a hemicolectomy was performed for colon cancer. The patient subsequently developed right-sided breast cancer, staged cT4b N1 M0, with bilateral opulent nodules on imaging. A thyroid nodule was identified as TI-RADS 4; fine-needle aspiration yielded an inadequate sample (67 % cellularity). The breast tumor was estrogen-receptor positive (8/8), progesterone-receptor negative (0/8), HER2-negative, and Ki-67 20 %. The treatment plan included neoadjuvant chemotherapy with weekly taxol, epirubicin (unavailable), adriamycin 100 mg, and cyclophosphamide (endoxan) 1000 mg, with close monitoring of cardiac function (EF 67 % on exam) and assessment of tolerability.

The patient returned for cycle 2 (C2) of adriamycin and endoxan, with supportive therapy; vital signs were weight 80 kg, height 155 cm and were stable.

For cycle 3 (C3) of adriamycin and endoxan, laboratory values remained stable (weight 80 kg, height 155 cm). A bone scan revealed a solitary osseous nodule in the right third rib, prompting MRI for further characterization.

Hemoglobin was 7.6 g/dL, prompting a blood transfusion and repeat CBC. Physical examination showed edema of the right breast, a 5 × 4 cm mass, a 2 × 2 cm right axillary lymph node, peau d'orange, and nipple retraction. A chest CT was ordered to assess the rib lesion.

The patient received week 1 (w1) of taxol 120 mg (70 mg/m²) despite poor performance status; tolerance was to be reassessed the following week.

Cardiology consultation was obtained after a decrease in ejection fraction to 37 % while on week 1 taxol 120 mg.

On examination during week 2 (w2) of taxol 120 mg, right-breast edema and induration were noted. Week 3 (w3) taxol was pending pending a repeat chest CT and a new echocardiogram after three weeks.

Due to a hemoglobin of 8 g/dL and a 10 % dose reduction, the patient received week 2 taxol 105 mg, one prophylactic dose of Neupogen, and a blood transfusion.

Because the total lymphocyte count was 1.4 × 10⁹/L (with an absolute neutrophil count of 0.4 × 10⁹/L), taxol was postponed for one week; if neutropenia persisted, the regimen would be deemed intolerable. Multidisciplinary discussion considered neoadjuvant hormonal therapy. Chest CT was clear, with a normal thoracic cage and no pulmonary nodules.

During week 4 (w4), examination revealed a right retro-areolar 3 × 3 cm mass with skin dimpling, nipple retraction, and a 1 × 2 cm right axillary lymph

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

node. Taxol 105 mg was administered, and prophylactic Neupogen was given every two weeks.

During week 5 (w5), the same right retro-areolar mass and skin changes persisted; taxol dose was reduced to 100 mg and two prophylactic doses of Neupogen were administered.

During week 6 (w6), examination showed a right axillary lymph node 2 × 2 cm and the retro-areolar mass unchanged at 3 × 3 cm. Taxol 100 mg and one prophylactic Neupogen were given; week 7 (w7) taxol was planned. The patient has now completed seven weeks of weekly taxol.

USTAVE/ OUSSY
ERNATIONAL

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جوستاف روسي
انترناشونال
مصر

ECHO REPORT

NAME : انشراح عزوز محمود DATE: 16 /3 /202

ID NUMBER : ٣١٩ /٢٥ SEX : ♀ AGE: ٢٨

F	67%	55-75%	PWT	1.0	0.6-1.1cm
/EDD	5.0	3.5-5.8cm	AO.R	2.9	3-3.7cm
/ESD		2-4cm	LA	4.5	2-4cm
S		25-45%	RV		0.7-2.3com(supine)
WT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ——— o.m.d

LV Wall Thickness ——— o.m.d

LV Diastolic Function Single Mitral (E) wave Absent A wave for ECG correlatio

RWMA ——— o RWMA

LA and Ao Root Dimensions Dilated LA

Mitral Valve ——— Mod MR

Aortic Valve Scl. AV with no sig systolic gradient with mild A.R

Tricuspid Valve ——— Mod TR ePASP 50 mmHg

Pulmonary Valve ——— o.m.d

Right Side and Systolic Function Dilated R.V with preserved systolic fendic

Intracardiac Masses and Effusion ———

DR

ECHO ECHO
[[DOCTYPE TEXT ECHO]] ECHO]]

STAVE / IUSSY

'atient Name	: الأستاذه / انشراح عزوز محمود	Request No.	: 13005398
'atient No.	: 24710030101624	Request Date	: 10/03/2026
ath No.	: C-48/26	Verification Date	: 11/03/2026
ex	: Female	Financial Category	: جوستاف روسي - مبادره
ge	: 78 Y	Department	: Out Patient
eferreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Neck U/S showing left thyroid lobe moderately suspicious nodule (TIRADs 4).
Nature of specimen: Left thyroid lobe nodule, U/S guided FNA, Cytology.

Gross specimen:
Four alcohol fixed smears were received & then they were H&E stained.

Microscopic examination
Adequacy: Inadequate smears.
Cellularity: Hypocellular.
Colloid: Present.
Follicular cells: Not presented.
Other cellular elements: Only red blood cells & lymphocytes.

Diagnosis
Left thyroid lobe nodule, U/S guided FNA, Cytology:
Unsatisfactory smears with no follicular cells, Bethesda category I.

Comment: The BETHESDA System for reporting thyroid cytopathology: Diagnostic categories:
I. Non-diagnostic or Unsatisfactory.
II. Benign. Consistent with a benign follicular nodule (includes adenomatoid nodule, colloid nodule, etc.).
III. Consistent with chronic lymphocytic (Hashimoto) thyroiditis in the proper clinical context.
IV. Consistent with granulomatous (subacute) thyroiditis.
V. Atypia of undetermined significance or follicular lesion of determined significance.
VI. Follicular neoplasm or suspicious for follicular neoplasm.
VII. Suspicious for malignancy.
VIII. Malignant.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

STAVE/
USSY
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Pathology Report

ignature
ır. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

الدكتور
حسين عبد العزيز يس
أستاذ الأشعة التشخيصية
كلية طب الأزهر

Name: Mrs. Ensherah Azouz Mahmoud.
Date: 18-Feb-26 8:41:56 PM Age: 78Y.
Dear Colleague Prof Dr.

Real time thyroid gland ultrasonographic study using superficial 10 MHz linear probe has revealed:

- The right thyroid lobe is enlarged in size shows large well defined mixed cystic and solid nodule with solid component is isoechoic, it measures about 3x3.5cm, no retrosternal extension.
- The left thyroid lobe and isthmus average of uniform homogenous echopattern and of smooth outline. Left thyroid lobe isoechoic solid nodule with macro calcifications, it measures about 15.5x16.5mm
- The thyroid gland measurements are:
  - Rt. lobe = 30 mm in AP diameter.
  - Lt. lobe = 20 mm.
  - Isthmus A.P. = 3 mm.

  The thyroid capsule is intact with no abnormal vascularity thyroid parenchyma using Doppler study.
- No evidence of retrosternal extension noticed.
- The neck vessels are of normal course and calibers.
- No evidence of suspicious lymphadenopathy noted.

Impression:

- ☒ Right thyroid non suspicious mixed cystic and solid nodule (TR2).
- ☒ Left thyroid lobe moderately suspicious nodule (TR4) for follow up.

Much Obliged, Yours,
Dr. Ayman Diab (MSC).

Sonographyography/Ultrasound
[[DOCTYPE SONOGRAPHY]]

Name : Aasharah Azzouz Mahmoud El-Laythi Date : 04-01-2026 10:44
Branch : Maadi Medical No. : 288141
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : Walkin

DIGITALIZED LOW DOSE SOFT TISSUE MAMMOGRAPHY WITH COMPLEMENTARY BREAST US EXAMINATION OF BOTH BREASTS REVEALED:

- ACR A: predominant fatty breast parenchyma.
- An ill-defined speculated hyperdense lesion is seen at the LOQ of the right breast corresponding by US to an ill-defined globular shaped heterogeneous hypoechoic lesion is seen at 6 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures 3x2.8 in its max dimensions.
- Few enlarged right axillary LNs one of them showing increased cortical thickness (>3mm) and effaced fatty hilum. It measures 1.3x1.3 cm.
- No pathological microcalcification clusters.
- No left breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- left non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Right breast UOQ malignant looking lesion associated with ipsilateral axillary lymphadenopathy (BIRADS V); for pathological correlation.**

Much Obliged;
Dr. Sarah Gamal, MSc.

Dr. Sara Gamal

Mammogram
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